

Riverside General Hospital

Patient Discharge and Follow-up Policy (Policy NUR-114, version 3.2)

Fictional sample document created for testing the RAG chatbot. All organisations, people, figures and contact details in it are invented.

1. Purpose and scope

This policy sets out how inpatients at Riverside General Hospital are prepared for discharge, how their care is handed over, and how they are followed up afterwards. It applies to all adult inpatient wards, including the Cardiology, Orthopedics, Oncology and Maternity units. Pediatric discharges follow the separate policy PED-020, and transfers to another hospital follow policy NUR-131.

2. Discharge criteria

A patient may be discharged when all of the following are true:

- The attending physician has documented that the patient is medically stable for discharge.
- Vital signs have been within the patient's target range for at least 12 hours.
- The patient, or their caregiver, can describe their medicines, warning signs and next appointment using the teach-back method.
- A safe destination is confirmed: home, a family member's home, or a care facility that has accepted the patient.
- Transport has been arranged, and the patient will not drive themselves after sedation.

Planned discharges should happen before 11:00 a.m. so that beds are ready for afternoon admissions. The hospital target is that at least 40 percent of discharges are completed before 11:00 a.m.

3. Discharge checklist

The discharging nurse completes the electronic discharge checklist in the patient record. The checklist must include:

- Medication reconciliation, completed by a pharmacist for patients who take five or more regular medicines.
- A written discharge summary, which must be sent to the patient's primary care provider within 24 hours of discharge.
- Removal of intravenous lines and urinary catheters, unless the care plan says they stay in place.
- Return of the patient's own medicines and valuables.
- Booking of all follow-up appointments before the patient leaves the ward.

4. Risk assessment

Every patient is scored with the Riverside Readmission Risk Score (RRRS) on the day before discharge. The score ranges from 0 to 20 and is calculated in the patient record from the patient's age, the number of admissions in the past 12 months, the number of chronic conditions, and whether the patient lives alone.

- Low risk: a score of 0 to 6.
- Medium risk: a score of 7 to 12.
- High risk: a score of 13 or above.

5. Follow-up timelines

Follow-up depends on the risk level recorded at discharge:

- High risk: a follow-up phone call from a transition-of-care nurse within 48 hours, and a clinic visit within 7 days.
- Medium risk: a follow-up phone call within 72 hours, and a clinic visit within 14 days.
- Low risk: a clinic visit or a telehealth appointment within 30 days if the physician requests one. No phone call is required.

If a high-risk patient cannot be reached after three call attempts on two different days, the nurse informs the patient's primary care provider and records the attempts in the patient record.

6. Readmissions

Any unplanned readmission within 30 days of discharge is reviewed by the Quality and Patient Safety Committee within 10 business days. The review checks whether the discharge criteria, the checklist and the follow-up timelines in this policy were met. The hospital's target 30-day readmission rate is below 12 percent for every adult ward.

7. Patient education

Education is given throughout the stay, not only on the day of discharge. Nurses use the teach-back method: the patient explains the instructions in their own words, and the nurse corrects any misunderstanding. Written instructions are provided in the patient's preferred language. Professional interpreters must be used for patients with limited English; family members should not interpret clinical instructions.

8. Roles and responsibilities

- Attending physician: decides when the patient is medically ready and signs the discharge order.
- Discharging nurse: completes the checklist and the teach-back education.
- Pharmacist: completes medication reconciliation where it is required.
- Case manager: arranges care facility placements, home health services and equipment.
- Transition-of-care nurse: makes the follow-up phone calls described in section 5.

9. Document control

- Policy owner: Director of Nursing Operations.
- Approved by: Chief Medical Officer, 14 February 2026.
- Effective date: 1 March 2026.
- Next review: March 2028. This policy is reviewed every 24 months.
- Replaces: version 3.1 (2024).